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REVIEW ARTICLE

Barriers to and Enablers of the Transition From Child to Adult Mental Health Services for Autistic Young People and/or Those With Attention Deficit Hyperactivity Disorder: A Scoping Review

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ABSTRACT

Background: Many autistic young people and/or those with attention deficit hyperactivity disorder (ADHD) have co-occurring mental health conditions. These individuals can experience significant challenges when transitioning from child and adolescent mental health services (CAMHS) to adult mental health services (AMHS). However, barriers and enablers to the transition from CAMHS to AMHS are poorly understood for this population. This scoping review sought to synthesise the available evidence on barriers and enablers to the transition from CAMHS to AMHS for autistic young people and/or those with ADHD.

Methods: Arksey and O'Malley's six-step framework for scoping reviews was used to map out the evidence available and report the findings with the Preferred Reporting Items for Systematic Reviews and Meta-Analyses extension for Scoping Reviews guidelines (PRISMA-ScR). Electronic databases of Medline, PsycINFO, CINAHL, Scopus and ProQuest Central were searched for relevant studies published in English with no date limitations. The review included studies that focused on young people with a primary diagnosis of autism spectrum disorder (ASD) and/or ADHD (population) and their care transition barriers and/or enablers (concept) from CAMHS to AMHS (context). A socioecological framework informed data extraction and narrative synthesis of data on barriers and enablers to transition.

Results: This review identified 1677 studies, of which 66 proceeded to full-text screening, and 10 were included. Most studies focused on institutional-level transition barriers, including poor communication between CAMHS and AMHS and lack of clarity about their role in the transition process. Involvement of the neurodivergent young person in decision-making processes about their mental health care and clinicians who tailored their approaches to the needs of the neurodivergent young person emerged as key enablers of a positive transition experience. Studies largely failed to examine community and policy-related barriers to mental health care transitions. Most studies examined the perspectives of providers or carers, with only five representing the perspectives of neurodivergent people.

Conclusion: Young people who are autistic and/or have ADHD experience many barriers to their transition from CAMHS to AMHS; however, few studies have examined barriers and enablers to the service transition process.

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Summary

- There are few studies reflecting the perspectives of autistic young people and/or those with ADHD on barriers and enablers to mental health care transition. Future research should prioritise eliciting their lived experience to enhance understanding of how to support these care transitions.
- Studies focused on institutional barriers and enablers to mental health service transition, largely neglecting community and policy-level barriers. To ensure a comprehensive understanding of mental health service transition barriers and enablers, future research should examine barriers at these levels.

1 | Introduction

Global changes over the past two decades have contributed to the significant increase in mental health conditions among young people between the ages of 12 and 25 (McGorry et al. 2024). For approximately 70% of this population, these mental health conditions will persist into adulthood, requiring continued support from mental health services (Cleverley et al. 2020; World Health Organization 2024). Mental health conditions are the leading cause of disability accounting for 45% of all disability adjusted life years among this age group and impacting on young people's potential to engage with their environment socially and economically (McGorry et al. 2024). Critically, neurodivergent young people are at elevated risk of mental health difficulties (Barber 2021). 'Neurodivergent' is an umbrella term used to describe a broad range of differences in neurocognitive functioning outside of societal norms. The term emphasises neurological variation as part of human diversity rather than pathology (Pellicano and Houting 2022). For autistic young people, and those with attention deficit hyperactivity disorder (ADHD), the risk of experiencing mental health difficulties, including anxiety and depression, is approximately three times higher than their neurotypical peers (Barber 2021; Pellicano and Houting 2022; Accardo et al. 2022). Consequently, neurodivergent young people are likely to be overrepresented in the population who need access to child and adolescent mental health services (CAMHS) (World Health Organization 2024; Accardo et al. 2022). Neurodivergent young people are more likely to require ongoing support from mental health services into adulthood due to persisting challenges to independently navigate all of life's changes during this period (World Health Organization 2024; Accardo et al. 2022). These transitions can include leaving school, starting a job or tertiary education or reductions in social supports (World Health Organization 2024; Accardo et al. 2022).

Many young people experience difficulties with the transition from CAMHS to adult mental health services (AMHS). The transition from CAMHS to AMHS typically occurs at 18 years of age, at which point the young person no longer meet the CAMHS eligibility requirements (Cleverley et al. 2020). CAMHS and AMHS differ in their expectations of young people and caregiver involvement in mental health care. As young people transition to AMHS, there is an expectation that they will assume greater autonomy for their mental health care and there will be less involvement from caregivers (Cleverley et al. 2020; Barber 2021). This service transition appears particularly challenging for neurodivergent youth who experience higher rates of care disruption and disengagement compared with their neurotypical peers (Cleverley et al. 2020; Barber 2021; Mulvale et al. 2019; Dunn 2017; Roughan et al. 2019; Cheak-Zamora and Teti 2015; McNicholas et al. 2015; Crane et al. 2019). Given the greater difficulties that neurodivergent youth experience with care transition, it is crucial that these barriers are understood from their perspective to help guide interventions to facilitate the transition from CAMHS to AMHS so they can continue to engage in the care they need (Cheak-Zamora and Teti 2015). At a time when the neurodivergent young person is often experiencing multiple simultaneous life transitions, disruption to service access to AMHS can increase vulnerability to mental health crises and increase difficulty in managing activities of daily living (Appleton et al. 2022).

Previous reviews have sought to explore the factors that influence care transitions for young adults with ADHD (Price et al. 2019; Rudgley 2013; Scarpellini and Bonati 2023; Swift et al. 2014), highlighting the limited availability of adult ADHD services, the young person's readiness for this service transition and (Price et al. 2019) and provider-related factors as barriers to the transition process (Rudgley 2013; Scarpellini and Bonati 2023). While these reviews provide important insight into care transition barriers, they all focus on ADHD. It is unclear whether these findings are also applicable to autistic young people. Although there are often overlapping features between ADHD and autism diagnoses, such as difficulties with executive function, behaviour and social skills, the nature and presentation of these features can differ (Mayes et al. 2012). One notable difference is that children with ADHD often have difficulty sustaining their focus, while autistic children can become hyperattentive to topics of interest to them (Mayes et al. 2012). Therefore, not all recommendations made for young people with ADHD will support the behavioural, social or cognitive needs of autistic young people (Coghill et al. 2023; Whitehouse et al. 2020).

This scoping review aimed to identify and synthesise the literature on barriers and enablers to the transition from CAMHS to AMHS for autistic young people and/or those with ADHD in order to inform intervention to facilitate care transition. This scoping review focused on autism and ADHD, the two most common forms of neurodivergence. To systematically identify gaps in the evidence base, we used Bronfenbrenner's socioecological model (SEM) as a conceptual framework (Bronfenbrenner 1976; Bronfenbrenner 1977; Kilanowski 2017). The SEM views mental health service use as being influenced by a range of interrelated factors relevant to the individual (e.g., beliefs), their interpersonal relationships (e.g., immediate social supports), their community (e.g., extended social supports), the health service organisations they engage with (e.g., their service policies and procedures) and broader mental health policies (Bronfenbrenner 1976; Bronfenbrenner 1977; Kilanowski 2017). The SEM is widely used to understand the barriers to a broad range of mental health interventions across different populations and settings (Thum et al. 2023; Wold and Mittelmark 2018; Jenkins et al. 2023; Tanhan and Francisco 2019; Scharpf et al. 2021).

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TABLE 1 | Sample search for Medline database used for this scoping review.

	Concept 1	Concept 2	Concept 3	Concept 4
Keywords	neurodiverse* or autism* or asperger* or ASD* or 'pervasive developmental disorder*' or PDD* or ADHD* or ADD* or 'attention deficit disorder*' or 'attention deficit hyperactivity disorder*'	'mental health services*' or 'child and adolescent mental health services*' or CAMHS or 'youth mental health services*'	transition* or transfer* or handoff* or 'hand off*' or handover* or 'hand over*'	teen* or youth* or adolescents* or 'young people*' or 'young adult*' or 'young person*'
Subject headings				
Medline (MeSH)	Autism Spectrum Disorder/or Asperger Syndrome/or Autistic Disorder/or Attention Deficit Hyperactivity Disorder/or Attention Deficit Disorder/ or Pervasive Developmental Disorder/or Neurodiverse/	Mental Health Services/	Transition to Adult Care/	Young Adult/ Adolescent/

2 | Methods

This review was informed by Arksey and O'Malley's framework for scoping reviews (Arksey and O'Malley 2005) and reported in accordance with the Preferred Reporting Items for Systematic Reviews and Meta-Analyses extension for Scoping Reviews (PRISMA-ScR) Checklist (Tricco et al. 2018). The protocol was registered with the Open Science Foundation (OSF) on 18/09/2023 (doi:/10.17605/OSF.IO/BZPQF) and published (Tang et al. 2024).

2.1 | Identifying the Research Question

We used the population, concept and context (PCC) framework to define the research question for this review. For this scoping review, the

i. population of interest were young people with a primary diagnosis of autism and/or ADHD;

ii. concept was barriers and enablers to the transition from CAMHS to AMHS;

iii. context was mental health services that provide psychotherapy support rather than medication only management.

More specifically, this review aimed to address the following research question: What are the barriers and enablers to the transition from CAMHS to AMHS for autistic young people and/or those with ADHD?

2.2 | Identifying Relevant Studies

We used a three-step approach to identify relevant studies. First, we developed an initial search strategy with the aid of a health sciences librarian that included search terms related to the PCC framework used for this scoping review. To ensure relevance, we sought feedback from a lived experience advisory group comprising members with lived experience of mental health services and who identified as being autistic and/or having ADHD. Members were not health practitioners. This advisory group provided input into terms to include in the initial search strategy such as previous neurodivergent diagnostic names, for example, Aspergers, pervasive development disorder and ADD.

This search strategy was piloted in Medline before being revised to include key words related to the PCC framework used to define the research question for this review as outlined in Section 2.1, as well as MeSH terms to yield more accurate results.

In the second step, one reviewer (K.T.) used the revised search strategy to conduct the database search. Medline (Ovid), CINAHL (EBSCO), PsycINFO (Ovid), ProQuest and Scopus Central electronic databases were searched for relevant studies on September 2023.

See sample search in Table 1.

Medline, CINAHL and PsychINFO databases were the three main data bases searched first, before searching for any additional relevant studies on ProQuest and Scopus Central databases.

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In the third step, the reference lists of all included studies were checked for additional studies not captured by the searches. SpiderCite (https://sr-accelerator.com/#/spidercite) was used to conduct forward and backward citation checks of included studies to identify any additional studies missed by the database searches. An updated search was conducted in August 2024 to identify relevant studies published between October 2023 and August 2024, with no additional relevant studies found.

2.3 | Study Selection

Study selection occurred in two stages between September and October 2023. After identifying articles from all five databases, articles were exported into Endnote and duplicates were removed. The list of deduplicated articles was extracted into an Excel spreadsheet and stored on a password protected drive through Curtin University in preparation for selection. During first stage screening, two reviewers (K.T. and P.W.) independently screened the titles and abstracts of all identified sources, using the Excel spreadsheet to record whether to include the article or exclude it from full-text review because the study focused on the incorrect population, context or concept. Articles that appeared relevant or where there was insufficient information about the population, context or concept to make an informed decision were progressed to full-text review for second stage screening. The same reviewers then assessed the articles that progressed to full-text review against predetermined inclusion and exclusion criteria, recording their responses and reasons for exclusion on an Excel spreadsheet.

Studies were eligible for inclusion if they looked at adolescents and/or young people (aged 18–24) with a primary diagnosis of autism and/or ADHD, who had experience with the transition process from CAMHS to AMHS. Studies were excluded if they mentioned other forms of neurodivergence that were not autism or ADHD (wrong population), did not explore the transition process from CAMHS to AMHS (wrong concept), only described the transition process for ongoing ADHD medication management (wrong context), did not report on individual study data, were not peer-reviewed, not written in English or could not be accessed.

Reviewer discrepancies were discussed with the rest of the research team for both the title and abstract and full-text screening stages and resolved by consensus. Intercoder agreement for title and abstract screening was almost perfect (Cohen's kappa = 0.94) and substantial (Cohen's kappa = 0.80) for full-text screening (Landis and Koch 1977).

Studies that met the inclusion criteria in the full-text review were included for data extraction.

2.4 | Charting the Data

A data charting sheet was developed and piloted to ensure it captured all relevant information. Two research team members then used this tool to independently chart all relevant study information from each included article between February and March 2024 (K.T. charted data from all 10 included articles, BM charted data from four articles, C.T. and E.H. both charted data from three articles). More specifically, these reviewers charted data on (1) study characteristics (authors, publication year, country, study setting, study design and sample size), (2) the population (age, gender, ethnicity and type of neurodivergence) and (3) reported barriers and enablers to the transition from CAMHS to AMHS. Any disagreements were resolved by discussion with all four team members until a consensus was reached.

2.5 | Collating, Summarising and Reporting Results

The SEM was used as a framework for guiding the narrative synthesis and reporting of these findings (Kilanowski 2017). We had initially proposed using Bronfenbrenner's SEM (Bronfenbrenner 1976) in conjunction with the Social-Ecological Model of Adolescent and Young Adult Readiness for Transition (SMART) framework (Schwartz et al. 2011). During data charting, we found that few elements of the SMART framework were represented in the charted data and those that were represented overlapped with the SEM. We therefore decided to only use the SEM (Thum et al. 2023) as a conceptual framework.

As none of the included studies explicitly referenced or applied the SEM, the research team met to categorise each charted barrier or enabler as an intrapersonal, interpersonal, community, institutional or policy-related factor in line with the components of the SEM (Kilanowski 2017). The definitions we used to categorise these barriers against the SEM levels are presented in Table 2.

The quality of the included studies was assessed using the Standard Quality Assessment Criteria for Evaluating Primary Research Papers from a Variety of Fields (hereafter referred to as the Standard Quality Assessment) (Kmet et al. 2004). Two authors assessed the quality of each paper, with quality defined as either strong (> 80%), good (70%–80%), adequate (50%–70%)

TABLE 2 | Definition of barriers and enablers at each level of the SEM levels used to guide data charting and the collation of results.

Intrapersonal	Factors that impacted the neurodivergent young person directly, for example, their beliefs or feelings about their transition process.
Interpersonal	Assistance provided by their caregivers to assist their transition process, for example, advocacy, driving them to their appointments.
Community	Social supports, for example, community resources
Institutional	Factors that indirectly impacted the neurodivergent young person through the mental health services, such as communication between child and adult mental health services.
Policy	Local and national guidelines that govern the way mental health services operate.

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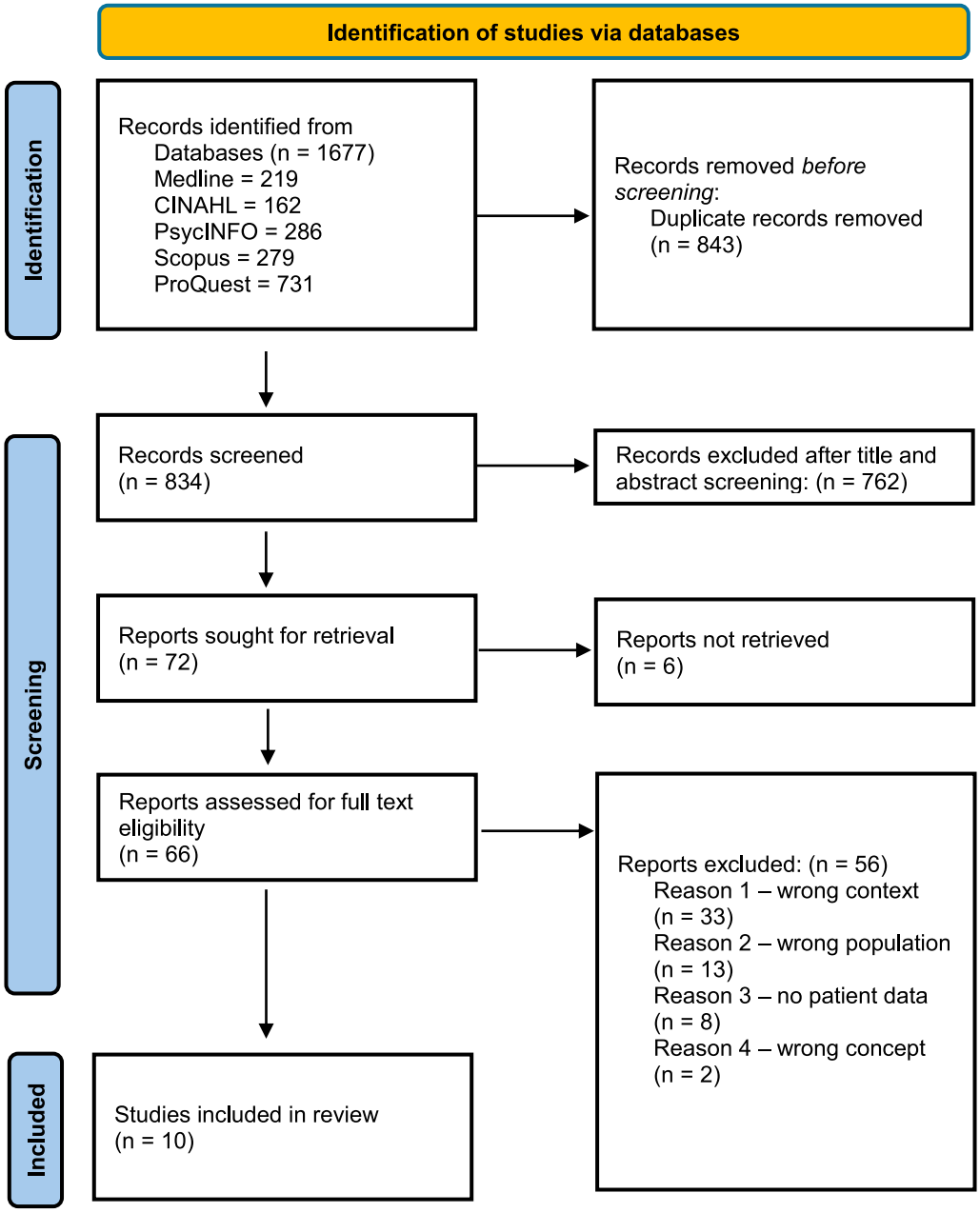


FIGURE 1 | PRISMA scoping review flow diagram of study selection for included studies in this scoping review (Cheak-Zamora and Teti 2015).

or limited (< 50%) (Lee et al. 2008). Reviewer discrepancies were discussed with the rest of the research team and resolved by consensus. Intercoder agreement was moderate (Cohen's kappa = 0.6) (Landis and Koch 1977).

3 | Results

Searches identified a total of 1677 studies. Once the 843 duplicates were removed, 834 titles and abstracts were screened and 72 progressed for full-text review. Of these, full texts sought for retrieval, 66 were found and assessed for full-text eligibility and six were unable to be retrieved by the two reviewers or with the aid of the health sciences librarian. Specific reasons for exclusion were not recorded at the title and abstract screening stage, however, was recorded at full-text screening stage with 56 studies being excluded for being the wrong context ($n = 33$), wrong population ($n = 13$), no patient data ($n = 8$) and wrong concept ($n = 2$). Ten studies that underwent full-text review met the inclusion criteria. See Figure 1.

Five studies utilised quantitative methodology (Hall et al. 2013, 2015; Merrick et al. 2020; Reis et al. 2021; Roberti et al. 2023), including cross-sectional and longitudinal cohort designs; four used qualitative methods (Newlove-Delgado et al. 2018; Price et al. 2022; Reale et al. 2015; Swift et al. 2013); and one study used mixed methods (Crane et al. 2019), see Table 3.

3.1 | Characteristics of Included Studies

Overall, the included studies examined care transitions from the perspectives of 275 neurodivergent young people, 24 parents and 259 health care providers (Crane et al. 2019; Hall et al. 2013,

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TABLE 3 | Summary of the study and population characteristics of included studies ($n = 10$) and their quality assessment rating.

Study characteristics								KMET rating
Study number	**Authors, (year), country**	**Study design**	**Participants**	**Age in years of young people (if reported)**	**Gender (if reported)**	**Ethnicity (if reported)[a]**	**Sample size**	**Quality assessment**
1	Crane et al., (2019), England	Mixed methods, online surveys and interviews	Autistic young people	16–25	Male = 32% Female = 51% Non-binary = 15% Prefer not to disclose = 2%	White = 88.5% Black = 0.8% Asian = 4.6% Mixed = 3.8% Other = 2.3%	130	Qual: good Quant: strong
2	Hall et al., (2015), England	Cross-sectional survey	Health care professionals providing services for young people with ADHD	N/A, as young people's views were not elicited	Not reported	Not reported	37	Good
3	Hall et al., (2013), United Kingdom	Cross-sectional survey	Health care professionals providing services for young people with ADHD	N/A, as young people's views were not elicited	Not reported	Not reported	96	Strong
4	Merrick et al., (2020), United Kingdom	Longitudinal cohort study	Autistic young people	14–18	Male = 69.5% Female = 30.5%	98.3% described themselves as white British	118	Strong
5	Newlove-Delgado et al., (2018), South West Peninsula of the United Kingdom	Qualitative, phenomenological study	Young people with ADHD	17–19	Male = 71% Female = 29%	Not reported	7	Adequate
6	Price et al., (2022), United Kingdom	Qualitative, phenomenological study	Health care professionals ($n = 15$ paediatricians; $n = 7$ psychiatrists)	N/A, as young people's views were not elicited	Not reported	Not reported	22	Adequate
7	Reale et al., (2015), Italy	Qualitative explorative study	Parents of young people with ADHD ($N = 24$) and child psychiatrists ($N = 27$)	N/A as young people's views were not elicited	Not reported	Not reported	51	Adequate
8	Reis et al., (2021), Brussels, Belgium	Cross-sectional survey	Autistic young people	17–18	Male = 70% Female = 30%[b]	Not reported	10	Adequate

(Continues)

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TABLE 3 | (Continued)

Study characteristics								KMET rating
Study number	Authors, (year), country	Study design	Participants	Age in years of young people (if reported)	Gender (if reported)	Ethnicity (if reported)[a]	Sample size	Quality assessment
9	Roberti et al., (2023), Italy	Cross-sectional survey	Health care professionals, on behalf of mental health services as a collective ($n = 42$ CAMHS, $n = 35$ AMHS)	N/A as young people's views were not elicited	Not reported	Not reported	77	Strong
10	Swift et al., (2013), Nottinghamshire	Qualitative	Young people with ADHD	17–18	Male = 80% Female = 20%	All were White British	10	Adequate

[a]Data extracted according to the paper.
[b]This paper reported on sex rather than gender.

2015; Merrick et al. 2020; Newlove-Delgado et al. 2018; Price et al. 2022; Reale et al. 2015; Reis et al. 2021; Roberti et al. 2023; Swift et al. 2013). Of the 10 studies, seven explored barriers to care transition for young people with ADHD (Hall et al. 2013, 2015; Newlove-Delgado et al. 2018; Price et al. 2022; Reale et al. 2015; Roberti et al. 2023; Swift et al. 2013), and three explored transitions for autistic young people (Crane et al. 2019; Merrick et al. 2020; Reis et al. 2021). Five studies collected data from young people, three of these studies enrolled autistic young people (Crane et al. 2019; Merrick et al. 2020; Reis et al. 2021) and two enrolling young people with ADHD (Newlove-Delgado et al. 2018; Swift et al. 2013). The remaining five studies collected data from health care professionals providing services for young people with ADHD (Hall et al. 2013, 2015; Price et al. 2022; Reale et al. 2015; Roberti et al. 2023), of which one also collected data from parents/caregivers (Reale et al. 2015). Sample sizes ranged from seven to 130 participants. Four of the five studies that reported on gender, comprised of predominantly male participants (Merrick et al. 2020; Newlove-Delgado et al. 2018; Reis et al. 2021; Swift et al. 2013). All three studies that reported on ethnicity were predominantly Caucasian participants (Crane et al. 2019; Merrick et al. 2020; Swift et al. 2013).

Four studies looked at the transition from government-funded CAMHS to AMHS (Hall et al. 2013, 2015; Price et al. 2022; Reale et al. 2015); one study examined the transition from a mix of government-funded, social and private mental health services (Newlove-Delgado et al. 2018); and the remaining five studies did not specify whether they examined government-funded or private services (Crane et al. 2019; Merrick et al. 2020; Reis et al. 2021; Roberti et al. 2023; Swift et al. 2013).

All these studies were conducted in Europe, seven of which were conducted in the United Kingdom (Crane et al. 2019; Hall et al. 2013, 2015; Merrick et al. 2020; Newlove-Delgado et al. 2018; Price et al. 2022; Swift et al. 2013), two in Italy (Reale et al. 2015; Roberti et al. 2023) and one in Belgium (Reis et al. 2021).

Notably, only three of the 10 included studies investigated barriers and enablers of the transition process (Crane et al. 2019; Hall et al. 2013; Merrick et al. 2020) as a primary research aim. For the other seven studies, this was only a secondary aim (Hall et al. 2015; Newlove-Delgado et al. 2018; Price et al. 2022; Reale et al. 2015; Reis et al. 2021; Roberti et al. 2023; Swift et al. 2013). Specific barriers and enablers identified by these studies are reported in Section 3.3.

3.2 | Quality Assessment of Included Studies

Of the 10 studies, five were rated as having adequate quality (Newlove-Delgado et al. 2018; Price et al. 2022; Reale et al. 2015; Reis et al. 2021; Swift et al. 2013), one as good quality (Hall et al. 2015) and three as strong quality (Hall et al. 2013; Merrick et al. 2020; Roberti et al. 2023). The qualitative component of the mixed methods study was rated as good quality and the quantitative component as strong (Crane et al. 2019). See Table 2. Studies that scored as 'adequate' were found to have unclear study designs, data analysis methods not sufficiently described, sample size not justified, conclusions not supported by results, lack of

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connection to a theoretical framework/wider body of knowledge and no verification procedure(s) to establish credibility.

3.3 | Barriers and Enablers to the Transition Process

The included studies identified a range of barriers and enablers across most levels of the SEM. The barriers and enablers are reported below according to the relevant SEM level. See Table 4.

4 | Discussion

This scoping review found a paucity of literature describing barriers and enablers to mental health service transition for neurodivergent youth, with only 10 studies meeting the inclusion criteria for this review. This small evidence base is a cause for concern given the urgent need to inform interventions that support ongoing access to mental health care as neurodivergent young people age out of CAMHS (Crane et al. 2019; Hall et al. 2013, 2015; Merrick et al. 2020; Newlove-Delgado et al. 2018; Price et al. 2022; Reale et al. 2015; Reis et al. 2021; Roberti et al. 2023; Swift et al. 2013). This is particularly important given that this service transition is likely to occur at a time when neurodivergent young people are also experiencing developmental and social transitions that can impact their mental well-being (World Health Organization 2024; Accardo et al. 2022). The limited evidence base and the variable quality of available evidence, as highlighted by this review, underscores the need for more research on this topic so that tailored and person-centred, community and institutional-level strategies can be developed to facilitate the transition from CAMHS to AMHS for neurodivergent young people.

4.1 | Key Findings of Barriers and Enablers to the Transition Process

Despite these concerns, this review nevertheless identified key intrapersonal, interpersonal, community and institutional barriers that need to be addressed to facilitate the transition process. Barriers identified included a lack of individual knowledge information provided to all key stakeholders about the young person's transition process, professional support on how to provide mental health support to autistic young people and/or those with ADHD and joint planning between all key stakeholders (Crane et al. 2019; Hall et al. 2013, 2015; Merrick et al. 2020; Newlove-Delgado et al. 2018; Price et al. 2022; Reale et al. 2015; Reis et al. 2021; Roberti et al. 2023; Swift et al. 2013). Conversely, enablers of the transition process also exist at the same levels and include strong client–therapist relationships advocating for the individual, clear communication on why the transition was necessary and transition process expectations to help prepare the individual discharge from CAMHS and access AMHS and caregiver involvement to help advocate for the young person where they were unable to communicate their needs (Crane et al. 2019; Hall et al. 2013, 2015; Merrick et al. 2020; Newlove-Delgado et al. 2018; Price et al. 2022; Reale et al. 2015; Reis et al. 2021; Roberti et al. 2023; Swift et al. 2013). Interestingly, these barriers are similar to those reported by studies examining care transitions for young people with chronic health conditions or disabilities (Terry 2023; Kennedy et al. 2022).

4.2 | Gaps in the Literature Identified by This Review

In addition, this review highlighted three important gaps in the literature that would benefit from further research. First, despite the recognition of positioning the neurodivergent young person at the centre of the service design and delivery to enhance service outcomes (Marshall-Lee et al. 2020), young people's perspectives have not been extensively explored in the existing literature. Where five of the studies reported on the lived experiences of the young person, it was a small sample size (7–130 participants) (Crane et al. 2019; Merrick et al. 2020; Newlove-Delgado et al. 2018; Reis et al. 2021; Swift et al. 2013). Efforts have been made to improve outcomes for young people leaving CAMHS through a co-produced transition preparation programme with young people, health professionals and researchers, with the aim to collaboratively work together to produce original material rather than just commenting on existing policy and practice guidelines (Dunn 2017). Although existing transition guidelines exist for autism and ADHD, respectively, such as NICE (National Institute for Health and Care Excellence 2024), no studies have sought to include the lived experiences of autistic young people and/or those with ADHD in the development of a transition framework. Furthermore, previous studies have not sought to compare the perceived barriers and enablers of the transition process between neurodivergent young people compared with neurotypical youth. Therefore, it is uncertain how similar or different the transition process experience is and how well already available resources cater to the needs of neurodivergent young people or what neurodivergent specific supports need to be developed to support them to continue engaging in their mental health care (Crane et al. 2019; Swift et al. 2013).

A second limitation of the existing literature is that none of the included studies explicitly sought to explore barriers and enablers to the transition process across the entire treatment ecosystem. Studies often only examined the perspective of a single group of stakeholders within the service system (e.g., providers or young people), with none exploring the perspectives of policymakers or other health system leaders. Unsurprisingly, studies identified few barriers and enablers at the community and policy level as the opinions of community services and those involved with the policy-making process were not consulted. Community factors such as access to community resources, positive relationships and social connections have generally been identified as critical predictors of successful transition in mental health literature (Eriksson et al. 2018). However, comparable findings within this body of literature are limited, therefore restricting the knowledge of how similar or different the community level barriers to the transition process are for young people who are autistic and/or have ADHD. While no studies included in this review explicitly highlighted policy-level factors, we identified one policy enabler, namely, the active involvement of mental health professionals who are knowledgeable and have direct experience working with the community in advocating, drafting and shaping legislation to meet the needs of marginalised communities (Gondek et al. 2017). The SEM highlights that the mental health

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TABLE 4 | Barriers and enablers identified in the included studies of this scoping review in accordance with the SEM (Bronfenbrenner 1976).

SEM model factors	Barriers reported in the included studies of this scoping review	Enablers reported in the included studies of this scoping review
Intrapersonal ($n = 7$ studies) (Crane et al. 2019; Hall et al. 2013; Merrick et al. 2020; Newlove-Delgado et al. 2018; Price et al. 2022; Reis et al. 2021; Swift et al. 2013)	Neurodivergent young people: i. Having a lack of knowledge on how to access AMHS (Reale et al. 2015; Reis et al. 2021; Swift et al. 2013). ii. Not feeling developmentally ready for AMHS (Crane et al. 2019; Newlove-Delgado et al. 2018; Swift et al. 2013). iii. Being dissatisfied with the service provided by AMHS (Crane et al. 2019; Swift et al. 2013). iv. Not knowing who to go to or where to access support if their caregiver was also experiencing their own difficulties at the same time (Crane et al. 2019; Reis et al. 2021).	i. One study identified autonomy in decision-making as an enabler, with some young people having the choice to decide whether they wanted to be referred to AMHS and continue engaging in mental health services if they needed ongoing support (Merrick et al. 2020).
Interpersonal ($n = 4$ studies) (Crane et al. 2019; Newlove-Delgado et al. 2018; Roberti et al. 2023; Swift et al. 2013)	Caregiver involvement with system navigation and service attendance was described as both a barrier and enabler. This was dependent on both i. the level of support given to the young person (Crane et al. 2019; Newlove-Delgado et al. 2018; Reale et al. 2015; Swift et al. 2013) and ii. the willingness of caregivers to work together with providers to facilitate the transition from CAMHS to AMHS (Roberti et al. 2023; Swift et al. 2013).	
Community ($n = 2$ studies) (Crane et al. 2019; Merrick et al. 2020)	i. Negative community stigma, having to fit into a certain stereotype and not being taken seriously (Crane et al. 2019).	i. The inclusion of neurodivergent young people during shared decision-making regarding their mental health treatment, their ability to access community resources and opportunities to develop and maintain positive relationships within the context of social, educational and employment sectors (Merrick et al. 2020).
Institutional ($n = 9$ studies) (Crane et al. 2019; Hall et al. 2013, 2015; Merrick et al. 2020; Newlove-Delgado et al. 2018; Price et al. 2022; Reale et al. 2015; Roberti et al. 2023; Swift et al. 2013)	i. Availability of AMHS (including delays to service access, service eligibility criteria that made it difficult for autistic young people to receive ongoing care, AMHS that did not accommodate neurodivergence) (Crane et al. 2019; Hall et al. 2013, 2015; Merrick et al. 2020; Reale et al. 2015; Roberti et al. 2023; Swift et al. 2013). ii. CAMHS providers not adequately preparing young people for the service transition (Hall et al. 2013; Newlove-Delgado et al. 2018; Price et al. 2022; Reale et al. 2015; Swift et al. 2013). iii. Lack of service integration between CAMHS and AMHS (including stakeholder meetings and client handovers) (Hall et al. 2013, 2015; Price et al. 2022; Reale et al. 2015; Roberti et al. 2023). iv. Lack of standard operating procedures to guide the transition process (Hall et al. 2013; Merrick et al. 2020; Reis et al. 2021; Swift et al. 2013). v. AMHS providers not understanding or being trained in how to work effectively with autistic young people and/or those with ADHD (Crane et al. 2019; Hall et al. 2013; Reale et al. 2015; Roberti et al. 2023).	i. Strong client–therapist relationships, which were strengthened when providers were knowledgeable about autism and/or ADHD and could tailor their approach to meet the needs of neurodivergent young people (Newlove-Delgado et al. 2018).
Policy ($n = 0$ studies)	None of the included studies identified policy-level and broader contextual factors that helped or hindered this service transition.	

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system is complex and multifaceted (Bronfenbrenner 1976), suggesting that it requires stakeholder collaboration to effectively meet the neurodivergent young person's needs. As none of the included studies specifically sought to elicit the perspectives of all key stakeholders and integrate these data, it is difficult to implement a sustainable change at a policy level within mental health systems, to support the transition process for autistic and/or ADHD young people.

A third limitation of the existing literature is that it is limited to European mental health systems. Most studies were conducted in the context of the UK National Health System (NHS), two in Italy and one based in Belgium. Both the United Kingdom and Italy have adopted an NHS model that is publicly funded, making it more accessible to their residents, in comparison with Belgium, which is a regulated market system (RMS) where they are 'mostly for-profit (outpatient doctors) or not-for-profit publicly funded private providers' (Nicaise et al. 2020, 27). Enablers of an NHS model may include more continuity of care and better chronic care delivery rather than private or mixed health systems; however, RMS may allow their providers to have better management autonomy, making it easier to change and implement institutional policies and procedures (Nicaise et al. 2020). As a result, findings from these studies should be generalised with caution as not all countries will have the resources or be equipped to implement the enablers highlighted or reduce the barriers at each SEM level identified to support the transition process from CAMHS to AMHS for neurodivergent young people.

While acknowledging the limited evidence available, this scoping review has identified some key barriers and enablers to the mental health service transition process for autistic and/or ADHD young people. This can enable mental health services to address these key barriers identified in this review by (1) exploring how their existing policies impact neurodivergent populations; (2) allocating their resources to support the needs of neurodivergent young people, for example, staff training on the differences of neurodivergence and to ensure health professionals can adapt their communication styles when working with neurodivergent young people; and (3) providing open communication channels to actively invite feedback from neurodivergent young people to share their experiences in mental health services and making necessary adjustments to ensure safety and well-being, to encourage ongoing involvement in their mental health care (Hudson et al. 2019).

4.3 | Limitations

This review has several limitations. First, we excluded studies within the grey literature, which could have provided more breadth to the review findings regarding what is known and available in research. The decision to exclude these studies was related to difficulties in (1) searching systematically and transparently due to web search limitations to accommodate full search strategies and (2) ensuring quality control, as the grey literature produced by organisations or individuals may lack detailed descriptions of the methodological approaches used or can have biases if it is an area of interest, which could impact the credibility of the results (Mahood et al. 2014). Second, while recognising the range of forms of neurodivergence, this scoping review only looked at studies that were specific to autism and/or ADHD due to their prevalence in the general population (Stenning and Bertilsdotter-Rosqvist 2021; Doyle 2020). Therefore, recommendations may not be generalised to other forms of neurodivergence. Third, studies were excluded if they were not in English or could not be accessed. As a result, the studies included in this review were limited in geographical location which may not accurately represent what has been researched in other parts of the world. Additionally, of the studies that included the gender or ethnicities of their sample, it was predominantly Caucasian male participants thus findings of this review may not accurately reflect the lived experiences of neurodivergent young people with diverse gender and ethnic identities.

5 | Future Directions

More high-quality research is clearly needed to represent the diversity of mental health in neurodivergent young people. The existing literature provides a restricted and incomplete picture of the factors influencing service transition from CAMHS to AMHS for neurodivergent young people. Future studies should examine the perspectives of policymakers related to CAMHS to AMHS service transition to understand how state or national policies influence the allocation of resources and accessibility to mental health services, which in turn affects how the service provides treatment to the young person in their care. While recognising that no two people are the same, no two health systems are the same either. Therefore, to better understand the similarities or differences between health systems and implement neurodiverse tailored recommendations, further research is needed within other countries.

6 | Conclusion

This scoping review only included 10 studies, demonstrating the limited available literature regarding the transition process for autistic young people and/or those with ADHD. Findings suggest that neurodivergent young people experience many institutional barriers to their transition to AMHS. However, the extent to which community and policy-level factors influence care transition for autistic young people and/or those with ADHD remains poorly understood. Efforts to identify the full breadth of barriers and enablers to care transition are critical to designing and implementing targeted interventions to promote optimal service transitions and facilitate continued mental health care for this population.

Author Contributions

Kim Tang: conceptualization, data curation, investigation, methodology, project administration, resources, validation, visualization, writing – original draft, writing – review and editing. **Craig Thompson:** conceptualization, data curation, project administration, supervision, validation, visualization, writing – original draft, writing – review and editing. **Elizabeth Hill:** conceptualization, data curation, project administration, supervision, validation, visualization, writing – original draft, writing – review and editing. **Elizabeth Pellicano:** visualization,

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writing – original draft, writing – review and editing. **Phoebe Wan:** data curation, validation. **Bronwyn Myers:** conceptualization, data curation, project administration, supervision, validation, visualization, writing – original draft, writing – review and editing.

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Ethics Statement

The authors have nothing to report.

Conflicts of Interest

The authors declare no conflicts of interest.

Data Availability Statement

Data sharing is not applicable to this article as no datasets were generated or analysed during the current study.

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